Thyroid pathology report

Case TCGA-FY-A3TY

Report for. Collected Date & Time: Result Name. Reference Range.

SURGICAL P. (Age: Service: Surgery. Physician (s) : Specimen (s) Received. A: Thyroid. B: Right paratracheal lymph node level 6. Pathologic Diagnosis. A. Thyroid, total thyroidectomy: Papillary thyroid carcinoma, classical type. Tumor location Right thyroid lobe. Tumor size: 4.2 cm in greatest dimension. Tumor extends into perithyroidal adipose tissue with focal. superficial involvement of attached skeletal muscle. Resection margins negative. No lymph vascular invasion identified. Left thyroid lobe benign. Mild lymphocytic thyroiditis. Tumor tissue obtained for. B. Right paratracheal lymph nodes (level 6) : Six lymph nodes, all negative for metastatic. carcinoma.

CANCER CHECKLIST:

THYROID: Procedure: Total thyroidectomy. Specimen Integrity: Intact. Specimen Size: Right lobe: 4.5 x 2.5 x 2.0 cm. Left lobe: 4.0 x 2.0 x 1.0 cm. Isthmus with pyramidal lobe: 5.5 x 1.5 x 0.7 cm. Specimen Weight : 21 g. Tumor Focality: Unifocal. Dominant Tumor: Tumor Laterality: Right lobe. Tumor Size: 4.2 cm. Histologic Type: Papillary carcinoma. Variant. Classical. Architecture: Classical. Cytomorphology Classical. Resection Margins: Uninvolved. 1. Tumor Capsule: Partial encapsulated. Lymph-Vascular Invasion Not identified. Extrathyroidal Extension: Present. Extent: Minimal.

PATHOLOGIC STAGE: pT3, pNO, pMX. NOTE: Information on pathology stage and the operative procedure. is transmitted to this Institution' S Cancer Registry as required. for accreditation by the Commission on Cancer. Pathology stage. is based solely upon the current tissue specimen being evaluated. and does not incorporate other relevant data. Pathology stage is. only a component to be considered in determining the clinical. stage and should not be confused with nor substituted for it. The exact operative procedure is available in the surgeon' S. operative report. Additional Pathologic Findings: Thyroiditis, mild. Primary Pathologist: Clinical History. Papillary thyroid cancer. done on thyroid nodule. Gross Description. A. Received fresh for. research sampling labeled "thyroid -. stitch at right superior pole" is an intact 21 gram total. thyroidectomy specimen with attached stitch marking the right. superior pole. The capsule is intact, maroon-tan. Attached to. the anterior right lobe is an unremarkable fragment of tan. skeletal muscle (2.5 x 1.5 x 1.0 cm). The right lobe (4.5 x 2.5. X 2.0 cm) is inked blue. The left lobe (4.0 x 2.0 x 1.0 cm) is. inked black. The isthmus with pyramidal lobe (5.5 x 1.5 x 0. 7. cm) is inked yellow. The specimen is serially sectioned from. superior to inferior. Occupying the majority of the right lobe. is a partially encapsulated, ill-defined rubbery nodule (4.2 x. 2.- x 2.- cin) with a heterogenous tan-yellow cut surface. The. nodule is <0. cm from the capsule surface. No other discrete. masses or suspicious areas are grossly identified. The remainder. of the specimen has a homogenous maroon-red cut surface. A. portion of the nodule is submitted for. research. Representative sections are submitted for light microscopy as. labeled: A1-A5 - nodule (right lobe) every other slice from. superior to inferior with fragment of attached muscle in A3; A6 -. isthmus; A7-A9 - left lobe (majority) from superior to inferior. (nine cassettes) The specimen is reviewed with Dr. B. Received in formalin labeled "level 6 right paratracheal. lymph node" is a 2.5 x 2.0 x 0.3 cm aggregate of six tan-yellow,. irregular soft tissues which are submitted in toto (one. cassotte). Microscopic Description. Microscopic examination has been performed on all slides. The. pathologic diagnosis encompasses the essential microscopic. findings of this case. Interpretation performed at.

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